



Royal Berkshire
NHS Foundation Trust

Medical Management of Miscarriage Protocol (CG621)

Up to 14 weeks gestation (CRL = 84mm)

Approval

Approval Group	Job Title, Chair of Committee	Date
Gynaecology Clinical Governance	Chair, Gynaecology Clinical Governance	17 th May 2024

Change History

Version	Date	Author, job title	Reason
4	Jan 2019	B Chohan, Consultant O&G	Reviewed in line with current NICE guidelines
4.1	Feb 2019	B Chohan, Consultant O&G	Live change as result of RCA – pg. 7 para added re: sending POC to Oxford if 3 rd consecutive miscarriage
5	Nov 2021	J Nesbitt ST6, A Chase Consultant O&G	Reviewed in line with NICE 2021 and MifeMiso trial
6	March 2024	A Chase, Consultant O&G	Biennial review – no changes

Other relevant corporate or procedural documents:

- [Management of First Trimester Miscarriage Protocol \(CG656\)](#)

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

This document is valid only on date printed

Page 1 of 13

Contents

1.0	Purpose	3
2.0	Scope	3
3.0	Roles and Responsibilities	3
4.0	Introduction	3
5.0	Medical management of Miscarriage as an inpatient:	8
6.0	Consultation Undertaken	9
7.0	Dissemination/Circulation/Archiving	9
8.0	Implementation	9
9.0	Training	9
10.0	Monitoring of Compliance	10
11.0	Supporting Documentation and References	10
12.0	Equality Impact Assessment	11
Appendix 1 - Flow chart for medical management of miscarriage as an OUTPATIENT:		12
Appendix 2 - Flow chart for medical management of miscarriage as an INPATIENT:		13

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

1.0 Purpose

To assist medical and nursing staff in the use of Mifepristone and Misoprostol for the medical management of miscarriage.

2.0 Scope

- Early Pregnancy Unit
- Early Pregnancy Unit Staff

3.0 Roles and Responsibilities

- Consultant Lead for Early Pregnancy Unit
 - To ensure that all junior doctors understand the content of the protocol and are able to follow the guidelines in ensuring that patients are managed sensitively and effectively through this process.
 - To work with lead nurse to ensure that the protocol is reviewed regularly and updated to reflect any changes to current guidance.
- Lead Nurse for Early Pregnancy Unit
 - Co-ordinate training of all staff to ensure that protocol is utilised effectively
 - To ensure that all patients who are diagnosed with an early miscarriage are given appropriate support and guidance through diagnosis and treatment
 - To work alongside Consultant Lead to review protocol regularly and update as necessary to reflect changes in current national guidance.
- All Doctors and Nurses working within the Gynaecology Unit
- All staff to provide competent, effective and compassionate care for women diagnosed with a miscarriage

4.0 Introduction

This protocol guides the use of mifepristone and misoprostol in the medical management of miscarriage before the 14th week of pregnancy (or CRL $\leq$ 84mm). Mifepristone is an anti-progestogenic steroid. It sensitises the myometrium to prostaglandin-induced contractions and ripens the cervix. Misoprostol is a synthetic prostaglandin analogue that induces uterine contractions resulting in the evacuation of uterine contents. Miscarriage between 14-16 weeks / CRL > 84mm should be seen in EPU and managed as per Maternity care pathway for less than 24-week spontaneous loss.

Please read this protocol in conjunction with the [Management of First Trimester Miscarriage Protocol \(CG565\)](#).

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

4.1 Indications:

4.1.1 Missed miscarriage

- Medical management **should be offered** as a treatment option in **addition** to offering expectant and surgical management (SMM).

4.1.2 Benefits of medical management for missed miscarriage:

- **Fewer patients (17%) require SMM compared to those undergoing** expectant management (30%).
- Fewer emergency admissions than expectant management.
- Avoids surgical / anaesthetic risks.
- Faster, **more predictable commencement** of bleeding (after administration of drugs) than expectant management for missed miscarriage. Bleeding usually starts within **24 hours** of **medical** management.

4.1.3 Incomplete miscarriage

- **Do not routinely offer** medical management for **incomplete** miscarriage in the first trimester. There is **no advantage** over expectant management (no difference in time to resolution)
- During **expectant** management of incomplete miscarriage, the miscarriage is usually complete (80%) within **3 days**.
- **This does not mean that medical management is contraindicated.** Some women will have a strong preference for medical management despite counselling. Under these circumstances discuss with the consultant on call for possible medical management.
- Use Mifepristone 200mg followed by 600mcg Misoprostol (x1 dose only) 48 hours later for medical management of incomplete miscarriage if required

4.1.4 Multiple pregnancy

- Multiple pregnancy may be eligible depending on gestation and size. To discuss with the consultant on call for case-by-case basis.

4.2 Contraindications to medical management:

1. Haemodynamically unstable.
2. Haemoglobin <10g/dl (bleeding risk).
3. Severe pain.
4. Pyrexia > 38°C (possible infected products of conception).
5. Suspected molar pregnancy.
6. Anti-coagulant therapy (bleeding risk).
7. Clotting disorders (bleeding risk).

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

8. Porphyria.
9. Mitral stenosis.
10. Severe asthma.
11. Chronic adrenal failure (Not for Mifepristone).
12. IUCD in-situ (coil must be removed before administration of misoprostol).

4.3 Information for Patients:

4.3.1 Pre-consent counselling.

- Medical management shortens the time to **commencement** of missed miscarriage but **not** the time to **completion**.
- Even during **successful** medical management, **light bleeding may persist** for up to 3 weeks.
- Expect **heavy bleeding and cramping pains** for 24-48 hours after misoprostol administration.

4.3.2 Mifepristone and Misoprostol side effects are fairly common and include:

- Nausea and vomiting - resolve within 2-6 hours.
- Diarrhoea - resolves within 24 hours.
- Pyrexia and fever - resolve within 12 hours.
- Skin rash - resolves within 2-3 days.

4.3.3 Off-label prescribing of both Mifepristone and Misoprostol.

- Mifepristone is **not licensed** (off-label) for medical management of miscarriage.
- It is licensed for labour induction for foetal death in utero.
- Misoprostol is **not licensed** (off-label) for medical management of miscarriage.
- It is licenced for other conditions such as treating gastric ulcers.
- The off-license use should be discussed in the consenting process and the doctor should document this and obtain the patient's **e-consent**.
- Explain that **off-label treatments are fairly common** (most medicines administered to children are off-label) in medicine and that misoprostol is **effective and safe** in selected patients and is **recommended by NICE**.

4.3.4 Further advantages/disadvantages compared to the alternative management options:

Medical management:

- Greater **side effect profile** than expectant and surgical management.
- No difference in the **duration of pain or bleeding** cf. expectant management.
- Longer **duration of pain** and bleeding than **SMM**.
- There is no difference in **infection rates** (1-3%) cf. expectant and surgical management.

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

- No difference in **future pregnancy rates** cf. expectant and surgical management.

4.3.5 Repeat Pregnancy Test

- A pregnancy test will be provided to carry out at home 3 weeks after medical management of miscarriage. The doctor should advise patients to contact EPU sooner if they are experiencing worsening symptoms.

4.4 Medical management of miscarriage pathway:

1. Counsel patients on the advantages, disadvantages and alternatives to medical management.
2. Provide information leaflets on miscarriage and medical management.
3. Give the patient **time to decide** and the **option** to return at a later date for treatment.

If the patient decides on medical management:

4. Provide the Medical Management of Miscarriage Patient Instruction leaflet.
5. Obtain **e-consent** for medical management, include:
 - Failure (approx. 17%) and need for **SMM**
 - Mifepristone and Misoprostol are **unlicensed** for this indication.
 - Side effects, pain, potential heavy bleeding.
6. Document:
 - Temperature
 - Blood Pressure
 - Pulse
7. Take blood for:
 - FBC
 - G&S
8. Prescribe the pre-arranged pack of medications, which include Mifepristone, misoprostol, codeine and a pregnancy test.

Anti-D should **not** be administered to women undergoing medical management of miscarriage under 12 week's gestation.

Dispense 200mg Mifepristone to take **orally** followed by **800µg Misoprostol** (600µg if incomplete miscarriage) for the patient to administer **vaginally at home 48 hours after Mifepristone**.

4.5 Advice to patients who decide on medical management

4.5.1 Misoprostol administration.

- Insert the misoprostol vaginally 48hrs after taking Mifepristone.
- Pass urine before inserting misoprostol with fingers or a tampon.

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

- Eat and drink as normal.
- Use sanitary towels rather than tampons after inserting misoprostol.
- Lie down for 30-60 minutes after vaginal insertion.
- **Reinsert** tablets vaginally if they fall out.
- The 800µg misoprostol tablets **can** be taken orally but may be **less** effective (NICE 2019); this is why we advise **vaginal** administration.
- Paracetamol +/- ibuprofen analgesia which can be bought Your pack will contain codeine 15mg or 30mg tablets
- If nausea and vomiting are severe, patient may call and return for anti-emetics

Misoprostol is an abortifacient - It must be made clear to patients to:

- **Not keep** misoprostol tablets or give them to anyone else.
- To **return** any **unused** misoprostol tablets to any pharmacy.

4.5.2 What to expect post-administration

- Make sure the patient is aware of our **24-hour contact number** (included in the patient information leaflet).
- A member of the nursing staff will call 7-9 days after starting treatment to check commencement of bleeding and well-being.
- If the patient describes moderate or heavy bleeding that has subsequently reduced significantly, it can be assumed that there is likely a complete miscarriage. Advise a repeat pregnancy test in 3 weeks and to contact EPU if the pregnancy test is positive.
- If the patient does not give a history suggestive of complete miscarriage offer a second dose of 800mcg Misoprostol. If she wishes for this, arrange for her to attend for this as soon as possible. Also discuss alternative options – expectant management and surgical management.

Disposal of products of conception (POC).

Please read the protocol and provide the leaflet on '*Sensitive Management of Pregnancy Tissue*'.

Blood clots may be disposed of as normal.

Patients may decide to:

- Bring the pregnancy remains to the EPU for histology and sensitive disposal (patient will need to give written consent for this).
- Organise a private burial or cremation.
- Legally bury POC on private land when adhering to the regulations.

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

If this miscarriage is the third consecutive miscarriage, offer for the POC to be sent to Oxford for cytogenetics if this has not already been done in the past. You must add 'Testing for Cytogenetics' to the consent form. If POC are being sent for cytogenetics, they must not be sent in formalin and some POC must still be sent for histology.

4.5.3 Advice on when else to call the EPU:

- Any concerns.
- Prolonged or severe side effects.
- Prolonged (>3 days) heavy bleeding (flow not subsiding).
- Haemorrhage (clots the size of a palm or soaking sanitary pads every 20 minutes).
- Symptoms of haemodynamic instability or infection.
- Persistent bleeding for **more** than 3 weeks.
- **Positive home pregnancy test after 3 weeks.**

4.5.4 Follow-up:

1. No routine follow-up unless clinically indicated.
2. Patient to be given 24-hour contact number.
3. If the **bleeding stops**, the patient should perform a home urine pregnancy test **3 weeks later** and advised to call EPU if the test is positive.
4. Reasons for follow up:
 - Patient requests SMM.
 - Bleeding persists for >3 weeks.
 - Positive home urine pregnancy test 3 weeks later.
 - Patient request.

5.0 Medical management of Miscarriage as an inpatient:

- Inpatient management should be offered to patients who request it or for those measuring greater than 10 weeks gestation (CRL > 31mm) due to the risk of heavier bleeding.
- A side room should be made available on Sonning ward for these patients to maintain privacy.
- The medical management of miscarriage pathway will be completed after e-consent is taken.
- 200mg Mifepristone should be given orally and an appointment time given to return to Sonning ward 36-48 hours later.

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

- MEWS observations should be taken on admission and continued every 4 hours. If any changes occur in the patient's clinical condition then the frequency of the MEWS must be reassessed.
- On admission 800 micrograms of Misoprostol vaginally will be administered by the patient, nurse or duty doctor.
- A further 800 micrograms of misoprostol vaginally will be administered after 3 hours if no products of conception are passed.
- Patients can be discharged once the products of conception are passed, bleeding is not heavy and observations are stable.
- If no products of conception are seen and the pain and bleeding have settled, the patient may be allowed home with an ultrasound scan in the EPU within 48 hours.
- If medical management fails surgical management may be offered.

6.0 Consultation Undertaken

- EPU Nursing staff
- Gynaecology Clinical Governance Committee – This includes: a patient representative, Gynaecology Operations Managers, Gynaecology consultants, Junior and middle grade medical staff.
- A copy of the protocol was circulated to all stakeholders for amendments prior to a scheduled clinical governance meeting.

7.0 Dissemination/Circulation/Archiving

- A hard copy will be kept in the EPU and on Sonning Ward.
- Protocol will be available on the Trust intranet Policy Hub (Gynae protocol section) and external website.
- The Trust Secretary will be responsible for archiving old versions of this document.

8.0 Implementation

Medical and nursing staff will be reminded of protocol by EPU Lead nurse and the Gynaecology consultants.

9.0 Training

There is no mandatory training associated with this procedure. If staff have queries about its operation, they should contact their line manager in the first instance.

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

10.0 Monitoring of Compliance

Aspect of compliance or effectiveness being monitored	Monitoring method	Individual or dept. responsible for the monitoring	Frequency of the monitoring activity	Group/committee which will receive the findings/ monitoring report	Committee/ individual responsible for ensuring that the actions are completed
Miscarriage management. Reasons for Complaints	Oversight of Juniors & Complaint reviews	EPU	Monthly	Gynaecology MDT/Governance	/ J Waite

The Trust reserves the right to amend its monitoring requirements in order to meet the changing needs of the organisation.

11.0 Supporting Documentation and References

1. Chu J, Devall A et al. MifeMiso trial; Lancet 2020
2. National Institute for Health and Clinical Excellence NICE. Ectopic pregnancy and miscarriage: Diagnosis and initial management (NG126); 2021.
3. FIGO 2017 guidelines for misoprostol use.
4. National Institute for Health and Clinical Excellence NICE. Preterm Labour and Birth (NG25); 2015.
5. Trinder J et al. BMJ 2006; 332(7552): 1235-38.
6. British National Formulary
7. Neilson JP et al. Cochrane Database (2010). Jan 20(1): CD007223.

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

12.0 Equality Impact Assessment

	Age	Sex	Disability	Race	Gender	Religion or Belief	Sexual Orientation	Marriage and Civil Partnership	Pregnancy and Maternity
Do different groups have different needs, experiences, issues and priorities in relation to the proposed policy/change proposal?	N/A		N/A	N/A		N/A	N/A	N/A	N/A
Is there potential for or evidence that the proposed policy/change will not promote equality of opportunity for all and promote good relations between different groups?	-		-	-		-	-	-	-
Is there potential for or evidence that the proposed policy will affect different population groups differently (including unintended discrimination against certain groups)?	-		-	-		-	-	-	-
Is there public concern (including media, academic, voluntary or sector specific interest) in potential discrimination against a particular population group or groups?	-		-	-		-	-	-	-

Name of Protocol: Management of 1st Trimester Miscarriage Protocol

Do different groups (age, disability, race, sexual orientation, gender, religion or belief) have different needs, experiences, issues and priorities in relation to the proposed protocol?

NO

Is there potential for or evidence that the proposed policy will not promote equality of opportunity for all and promote good relations between different groups (age, disability, race, sexual orientation, gender, religion or belief)?

NO

Is there potential for or evidence that the proposed policy will affect different population groups (age, disability, race, sexual orientation, gender, religion or belief) differently (including possibly discriminating against certain groups)?

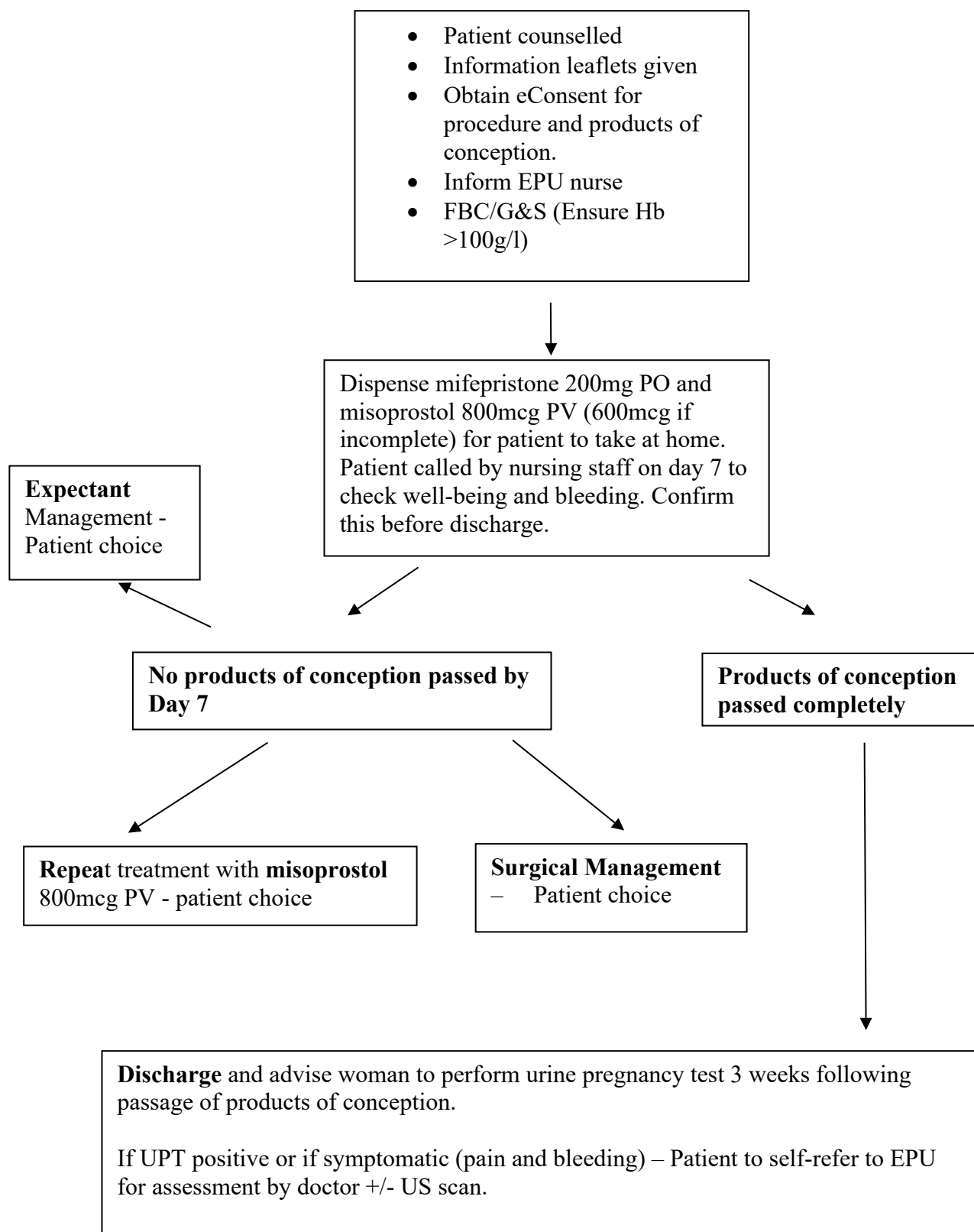
Affects women of reproductive age only

Is there public concern (including media, academic, voluntary or sector specific interest) in potential discrimination against a particular population group or groups (age, disability, race, sexual orientation, gender, religion or belief)?

NO

Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

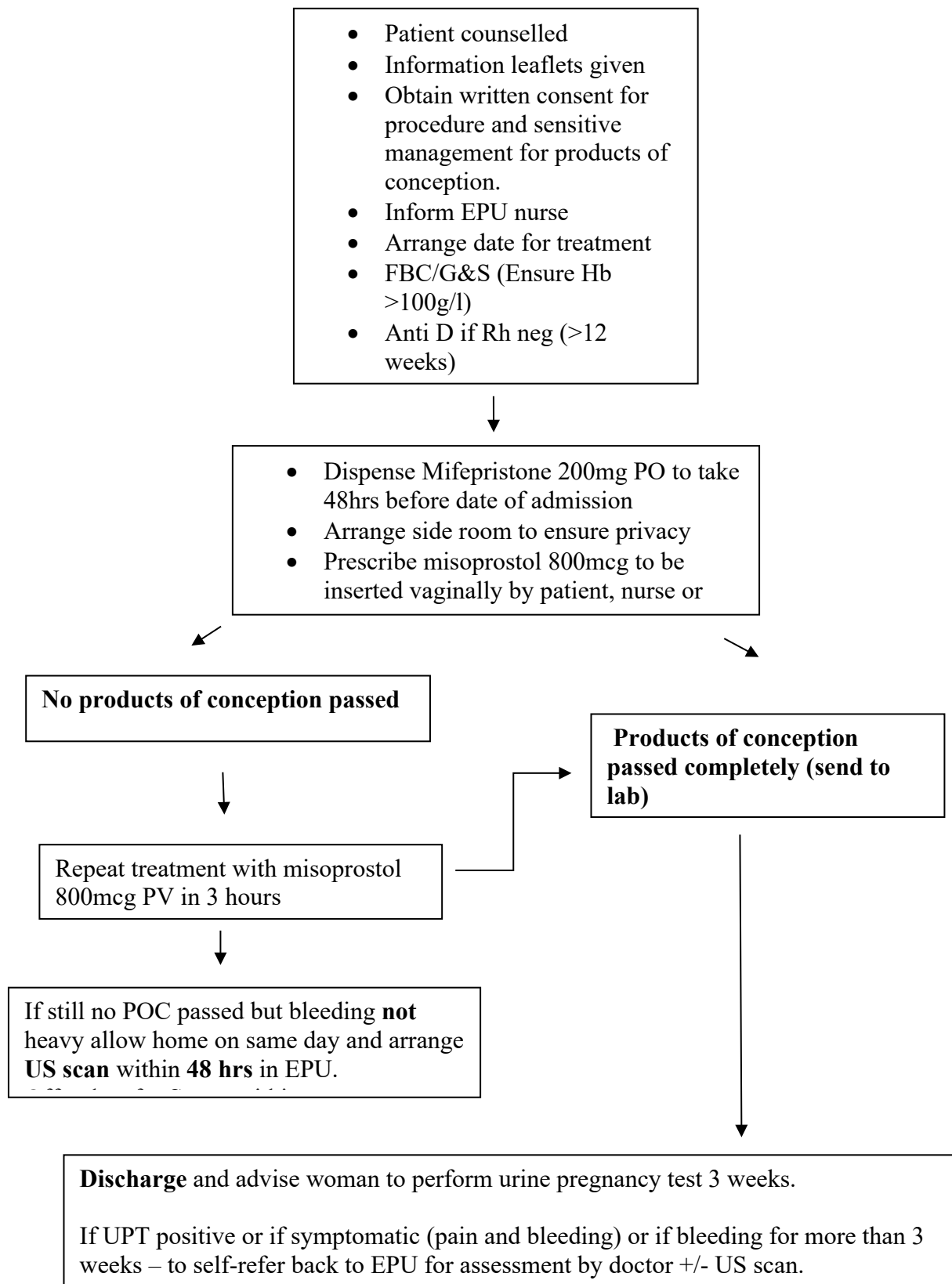
Appendix 1 - Flow chart for medical management of miscarriage as an OUTPATIENT:



Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		

Appendix 2 - Flow chart for medical management of miscarriage as an INPATIENT:

- Recommended for gestations > 10 weeks gestation



Author:	A Chase	Date:	May 2024
Job Title:	Consultant O&G	Review Date:	May 2026
Policy Lead:	Group Director Urgent Care	Version:	V6
Location:	Policy hub/ Clinical/ Gynaecology/ CG621		